

Meadowbrook Community Hospital

DISCHARGE SUMMARY

Patient: Dorothy Whitfield
DOB: 11/07/1954 (Age 71)

MRN: MRN-2025-0391

Admission Date: 08/15/2026
Discharge Date: 08/21/2026
Attending Physician: Dr. Elena Vasquez, MD (Pulmonology)

ADMISSION DIAGNOSIS:

1. Acute on chronic hypercapnic respiratory failure (J96.11)
2. COPD exacerbation with community-acquired pneumonia (J44.1)
3. Severe COPD, GOLD stage III
4. Medication lapse - tiotropium non-refill (contributory factor)

HOSPITAL COURSE:

71-year-old female with severe COPD admitted via ED with somnolence and critical hypercapnia (pH 7.21, pCO₂ 88). Managed successfully with BiPAP, avoiding intubation, consistent with her documented goals of care. Treated for COPD exacerbation with corticosteroids and bronchodilators; pneumonia culture-positive for H. influenzae, treated with targeted oral antibiotics after initial IV empiric therapy. Transferred to floor day 2, ambulating with supplemental oxygen by day 3. Home oxygen qualification testing completed - qualifies for 24-hour supplemental oxygen at discharge setting of 2L.

CONDITION AT DISCHARGE:

Stable on room air exercises with exertional desaturation managed by prescribed oxygen. No dyspnea at rest. Ambulating 150 feet independently.

DISCHARGE MEDICATIONS:

1. Prednisone 40 mg PO daily x 5 days (completes taper)
2. Amoxicillin-clavulanate 875/125 mg PO BID x 7 total days (day 4 of course)
3. Tiotropium 18 mcg inhaled daily (RESTARTED - critical maintenance medication)
4. Budesonide/formoterol inhaler 2 puffs BID
5. Albuterol HFA PRN q4h
6. Theophylline 300 mg PO daily (level checked at follow-up)

ALLERGIES: No known drug allergies

OXYGEN ORDERS:

- Oxygen 2 L/min continuous via nasal cannula; 4 L/min with exertion

- DME supplier arranged; conserving device provided

FOLLOW-UP:

- Pulmonology clinic 08/28/2026 - theophylline level, ABG reassessment
- Home health respiratory nursing visits 3x/week x 2 weeks
- Pulmonary rehabilitation referral submitted
- Smoking cessation counseling continued (quit 2019 - relapse prevention)

DISPOSITION:

Discharged to home; neighbor support network confirmed with patient consent.
Return precautions including increased somnolence or sputum change reviewed.